

DEE ROUNDTABLE 2026

Methodology Report

Companion to the Final Report

A Shared Vision for the Future: Rare and Complex Epilepsies in Australia



Contents

1	Purpose of Document	3
2	Project Design and Facilitation Approach	3
	Phase 1	3
	Phase 2	4
3	Stakeholder Selection and Sector-Balanced Representation	5
4	Pre-Engagement Activities: Briefings, Surveys and Evidence Packs	6
5	Data Collection Methods	8
	Morning session	8
	Afternoon session	11
	The final exercise	12
6	Appendices	13
	Appendix 1 Graphic recording, Workshop 1: Current situation	14
	Appendix 2 Graphic recording, Workshop 2: Desired and ideal situation	15
	Appendix 3 Graphic recording, Workshop 3: Barriers and drivers	16
	Appendix 4 Graphic recording: The shared vision	17

This document is a companion to *A Shared Vision for the Future: Rare and Complex Epilepsies in Australia*, the DEE Roundtable 2026 Final Report. Section numbers and colours follow the same code in both documents.

1 Purpose of Document

This document is a companion to the DEE Roundtable 2026 Final Report. It sets out the full design, facilitation approach, stakeholder selection process and data collection methods behind the Roundtable's findings and mandate. It is intended for the Steering Committee's own record, for anyone assessing the legitimacy of the process behind the mandate, and to support any future academic publication of this work, in the tradition of the 2019 Roundtable's publication in the *European Journal of Paediatric Neurology* (Kelada et al. 2025).

2 Project Design and Facilitation Approach

PHASE 1

The DEE Roundtable 2026 Project commenced with analysis of the DEE landscape and development of stakeholder engagement and communications plans. These laid the foundation for a strongly collaborative and representative approach throughout the project.

We then met with Childhood Dementia Initiative, Mito Foundation and Rare Cancers Australia to develop our understanding of how other organisations in Australia effectively represent people living with various diseases, of diverse genetic origin, in the manner of a peak body.

Our attention then turned to engaging with parents or carers of a child or children with a DEE. Parents or carers were first invited to respond to a questionnaire that sought to understand their experiences, priorities, challenges and ideas for change in relation to:

- When their child first experienced symptoms
- Searching for and/or receiving a diagnosis
- Receiving care by medical professionals
- Receiving treatment/s
- Their and their child's whole life, including their vision

Following analysis of responses and development of a discussion paper to support participants, parents or carers were invited to participate in a focus group, which explored:

- How families and professionals could work together differently to create more effective support systems
- In the context of research, policy or service design, questions or issues participants would want decision-makers to address
- What meaningful progress for the DEE community might look like, and what first steps could be taken together to get there

PHASE 2

Planning for the Roundtable itself then commenced in earnest, including development of a diverse and representative invitation list, selection of an experienced and trusted facilitator, consultations with key stakeholders to explore more complex matters in advance, and development of a discussion paper designed to support attendee preparation for the DEE Roundtable 2026.

The DEE Roundtable 2026: From Insights to Impact was a full-day event held in Brisbane on 1 May 2026. The full-day event was guided by independent facilitator Barrie Littlefield OAM and structured around a series of collaborative discussions. Attendees examined the complex challenges facing the DEE community, identified key barriers to progress, and worked towards shared priorities for action.



Figure 1. Group discussion during a workshop. Each group nominated a scribe to record the conversation on a flipchart and a presenter to summarise it for the room.

3 Stakeholder Selection and Sector-Balanced Representation

With reference to the stakeholder engagement planning completed earlier in the project, the Roundtable Steering Committee developed a comprehensive list of key opinion leaders in the Australian DEE landscape. With careful consideration given to balancing factors such as stakeholder type, institution and geographical spread, invitations were distributed to almost 100 key opinion leaders, and the extensive networks of the Steering Committee were leveraged to encourage attendance.

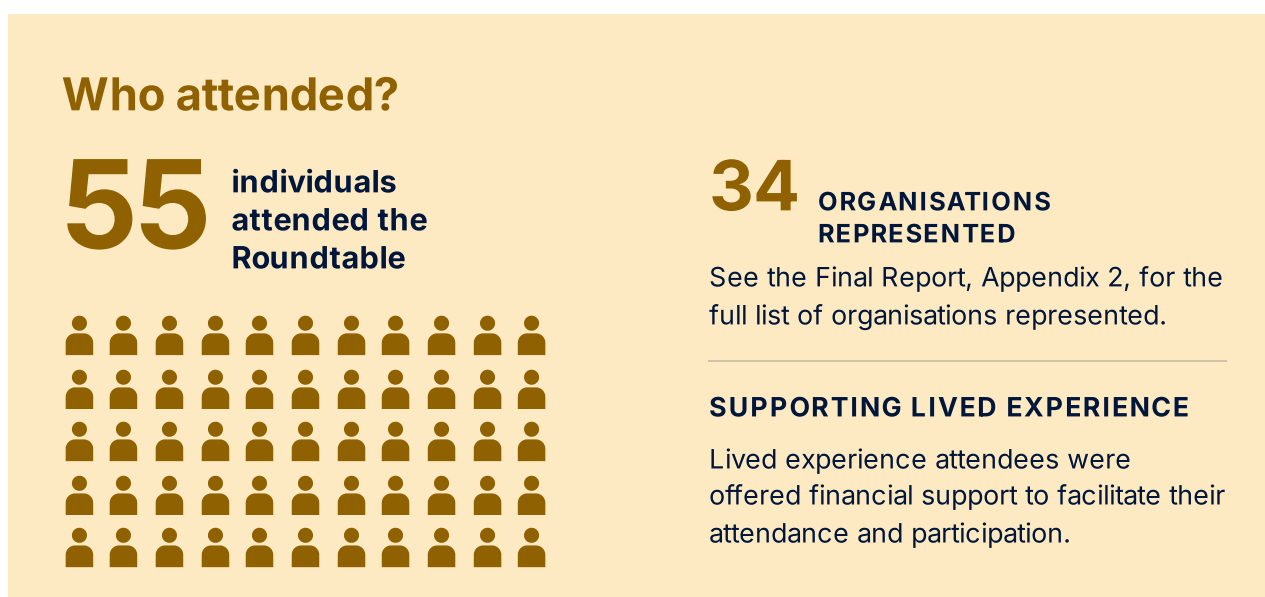


Figure 2. Attendance at the DEE Roundtable 2026, Brisbane, 1 May 2026.

Careful attention was paid to the curation of stakeholder balance in each discussion throughout the day, to maximise the diversity of perspectives and increase the consistency of outcomes. This was achieved using several seating plans, each including one Steering Committee member to provide guidance on how to conduct the exercises, and minimised or eliminated grouping of individuals from the same stakeholder group or institution.

4 Pre-Engagement Activities: Briefings, Surveys and Evidence Packs

Invitees were first sent a "Save the Date" email describing the project and the objectives of the Roundtable in the first working week of January 2026. The formal invitation was sent in March, accompanied by an Eventbrite page that included additional details and frequently asked questions.

In preparing the discussion paper, complex matters were identified for targeted exploration with key stakeholders.



Figure 3. Themes identified for targeted exploration with key stakeholders while preparing the discussion paper.

One of these key stakeholder groups is what we referred to as 'hard-to-reach' DEE families, an imperfect description that seeks to acknowledge the significant burden held by caregivers of individuals with DEEs, and the resultant challenge in engaging with these caregivers. As a result of this burden, they may be less engaged in consultation activities, less engaged in consumer advocacy, and less health literate, ultimately making it more challenging to hear their important voices.

The significant impact of caring for an individual with DEE on daily life is recognised in the literature (for example, Jensen et al. 2017, Kelada et al. 2025) and deeply felt in the DEE community. In acknowledgement of this burden on families, a consultation invitation was extended to epilepsy organisations from across Australia to support our understanding of those organisations' experiences in working with hard-to-reach segments of the DEE population.

COMMUNITY ORGANISATIONS CONSULTED

Epilepsy Western Australia	Epilepsy Action Australia
Epilepsy Foundation	Epilepsy Queensland
Epilepsy Tasmania	The Epilepsy Centre (The Epilepsy Association of SA & NT)

Key individual stakeholders were also consulted prior to the Roundtable. These conversations were pivotal in shaping the discussion paper and informing later stages of agenda planning.

Finally, pharmaceutical industry representatives, including project sponsors, were sent additional information via email the week prior to the Roundtable, communicating our expectation that sponsors attending the Roundtable would have the opportunity to:

- Contribute to shaping the future direction of the DEE landscape in Australia
- Strengthen relationships with clinicians, researchers, advocates and policymakers
- Gain direct insights from lived experience leaders
- Understand emerging priorities that will influence research, clinical trial readiness, access pathways and policy development

This was supported by a brief virtual conversation with major sponsor Lundbeck Australia Pty Ltd that expanded upon these opportunities, and a similar conversation with the representative of Alexion Pharmaceuticals, with a stronger focus on supporting their understanding of the DEE landscape, given that organisation's current focus on broader rare disease policy issues.

The central provocation of the discussion paper was that rare and complex epilepsies, including DEEs, must be treated as a distinct priority group rather than a subset of general epilepsy, and that a fundamentally different, person-centred and system-wide response is urgently required.

It presented examples of effective multi-stakeholder collaboration from Australia and internationally, and painted a picture of the Australian DEE landscape.

5 Data Collection Methods

On the advice of the independent facilitator, the Roundtable was divided into morning and afternoon sessions.

MORNING SESSION

The morning session comprised three workshops of around 60 minutes each. For each workshop, attendees were assigned to a different group of approximately eight to nine individuals, ensuring stakeholder diversity and balance in each conversation. Each workshop involved approximately 30 minutes of group discussion, in which a group-nominated scribe recorded a summary of the conversation on a large flipchart.

The second half of each workshop involved a group-nominated presenter sharing the flipchart and major discussion points with all attendees; each group had approximately five minutes to provide a verbal summary.

Workshops 1 and 2 explored the Current Situation (where are we now?) and then the Desired / Ideal Situation (where do we want or need to be in five years' time?), respectively, from four different perspectives:

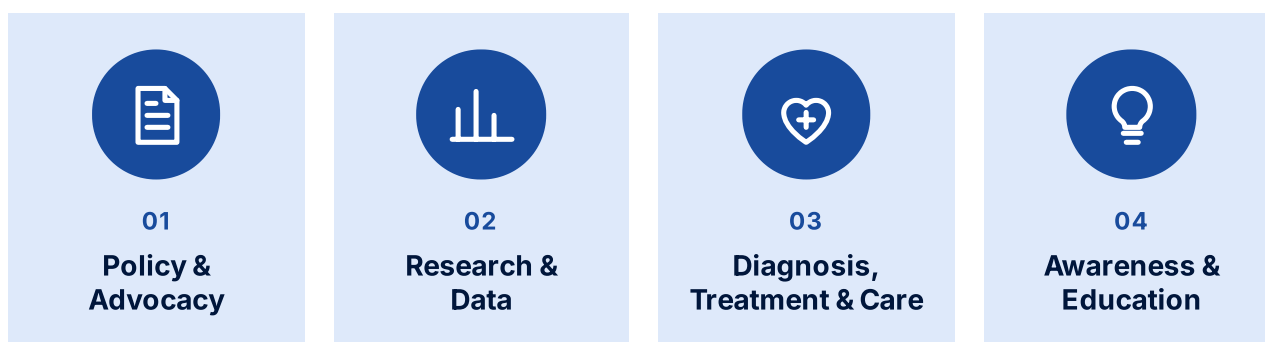


Figure 4. The four perspectives used to structure Workshops 1 and 2.

In Workshop 3, half of the groups explored the Barriers to moving from the current to the desired / ideal situation (what is stopping us, or will stop us, from getting there?), while the other half explored Drivers (what is helping us get there, or what do we need to get there?).

During the first half of each workshop, the independent facilitator and Project Manager circulated among groups, available to answer questions and provide gentle guidance to attendees as needed or upon request. Rachel Apelt of artbalm was engaged to provide graphic recording services for the day, listening to conversations to gain a preliminary sense of themes.

During the second half of each workshop, Rachel graphically recorded the verbal presentations, with reference to the flipcharts to support understanding of terms as needed. Attendees were able to watch these recordings develop live throughout the session. Photographs of each flipchart were also taken, to capture each written point.



Figure 5. Pages 1 and 2 of the graphic recording: the current situation and the desired / ideal situation. Reproduced at full size in Appendices 1 and 2.

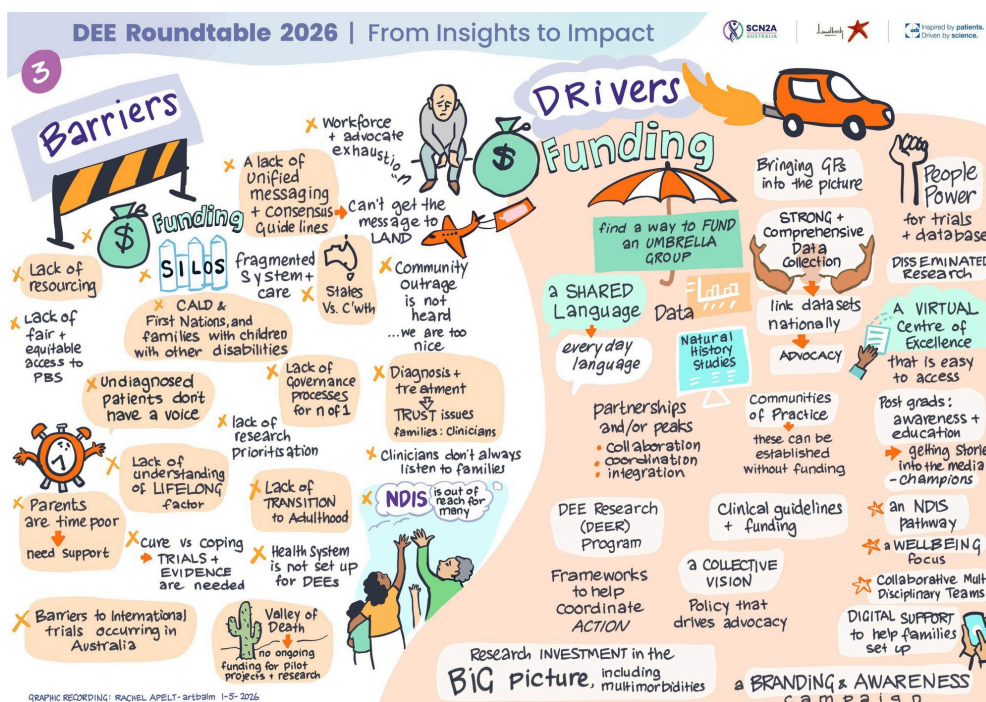


Figure 6. Page 3 of the graphic recording: barriers and drivers. Reproduced at full size in Appendix 3.

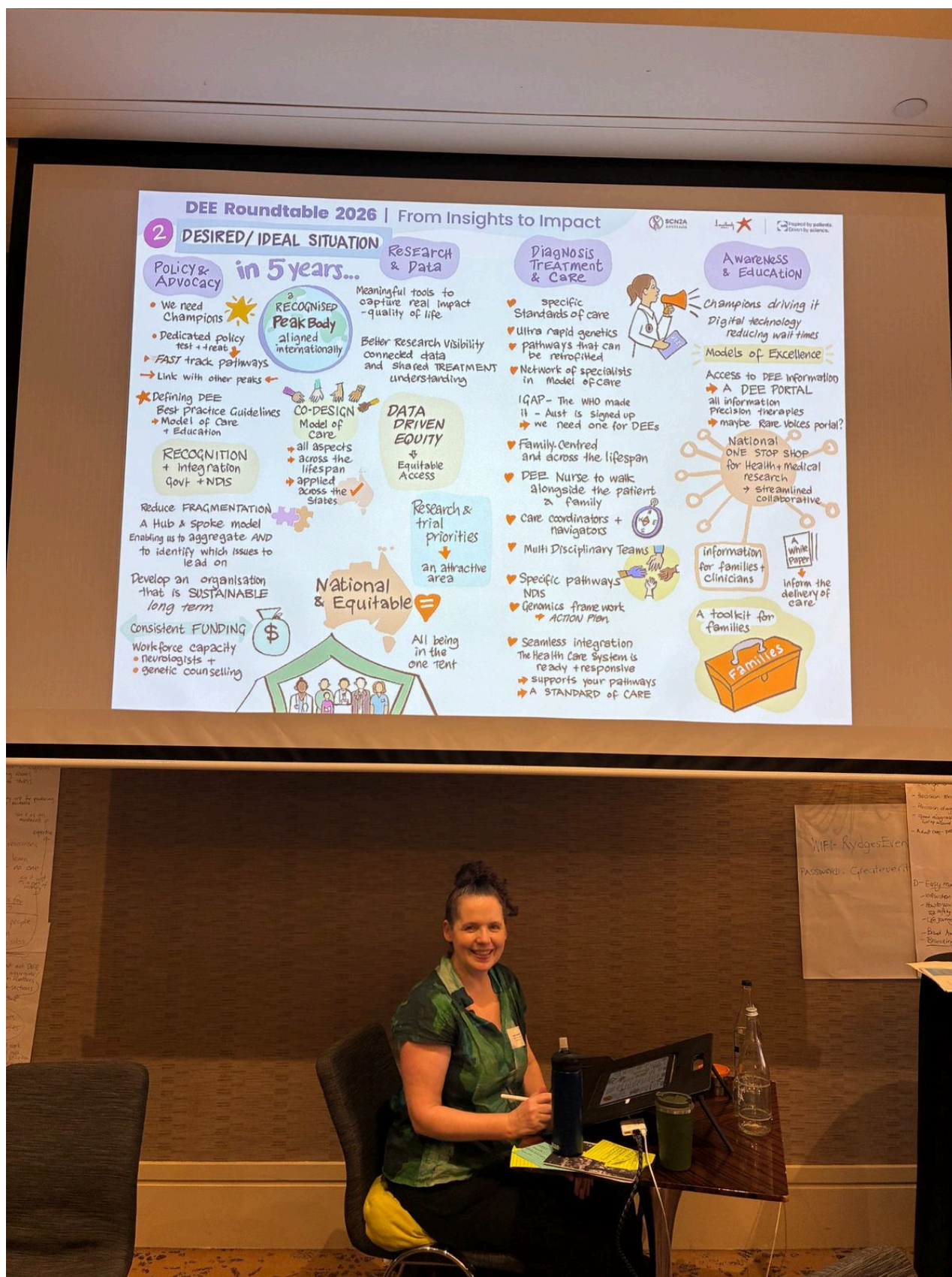


Figure 7. Rachel Apelt of artbalm developing the graphic recording of Workshop 3. Attendees watched each recording take shape live as the verbal summaries were given.

AFTERNOON SESSION

Building on the momentum gained throughout the first part of the day, Workshop 4 had a focus on Goals. First, each attendee had the opportunity to articulate their three top-most sector goals on a sticky note, in duplicate. Next, each individual shared their top three goals with their table group, comprising approximately eight or nine diverse stakeholders. One sticky note for each articulated goal was then posted on a whiteboard in the main room, and photographs were taken to capture each goal identified.



Figure 8. Left: every goal articulated, posted on the whiteboard in the main room. Right: a table group agreeing its top three goals.

Each table group was then asked to agree on their top three goals of the approximately 24 to 27 options discussed in their group. Once agreed, each table transcribed its top three goals onto a flipchart, which was then attached to a long wall. Photographs were taken of each flipchart to record all 21 top goals (three goals from all seven tables).

Each attendee was supplied with two red stickers and asked to use these to indicate their top one or two priority goals. Similarly, each attendee was supplied with two blue stickers and asked to use these to indicate the goals they felt might be 'low-hanging fruit'; that is, relatively quick and easy to achieve.



Figure 9. The prioritisation exercise. Red stickers marked priority goals; blue stickers marked goals judged quick and easy to achieve.

Attendees were able to see the results of these prioritisation exercises as they emerged. Rachel also recorded the results.

THE FINAL EXERCISE

The final exercise built on the strong theme that had emerged throughout the day: to establish an organisation to represent all stakeholders from across the Australian DEE landscape, responsible for executing the goals articulated to achieve the future state envisioned. It allowed attendees to discuss, within their table groups, ideas for the vision and mission of this organisation, empowering sector-wide involvement in its establishment. Vision and mission statements were written out for each table, and then read to the room. This facilitated recording of key themes by Rachel.

6 Appendices

The graphic recording of the DEE Roundtable 2026 was produced live on the day by Rachel Apelt of artbalm, Brisbane, 1 May 2026. It is reproduced here at full page size as a primary record of the workshops. The four pages appear in the order the workshops were run.

Appendix 1 Graphic recording, Workshop 1

The current situation: where are we now? Recorded from the four perspectives of policy and advocacy, research and data, diagnosis, treatment and care, and awareness and education.

Appendix 2 Graphic recording, Workshop 2

The desired and ideal situation: where do we want or need to be in five years' time? Recorded from the same four perspectives.

Appendix 3 Graphic recording, Workshop 3

Barriers and drivers: what is stopping us, and what is helping us, in moving from the current to the desired situation.

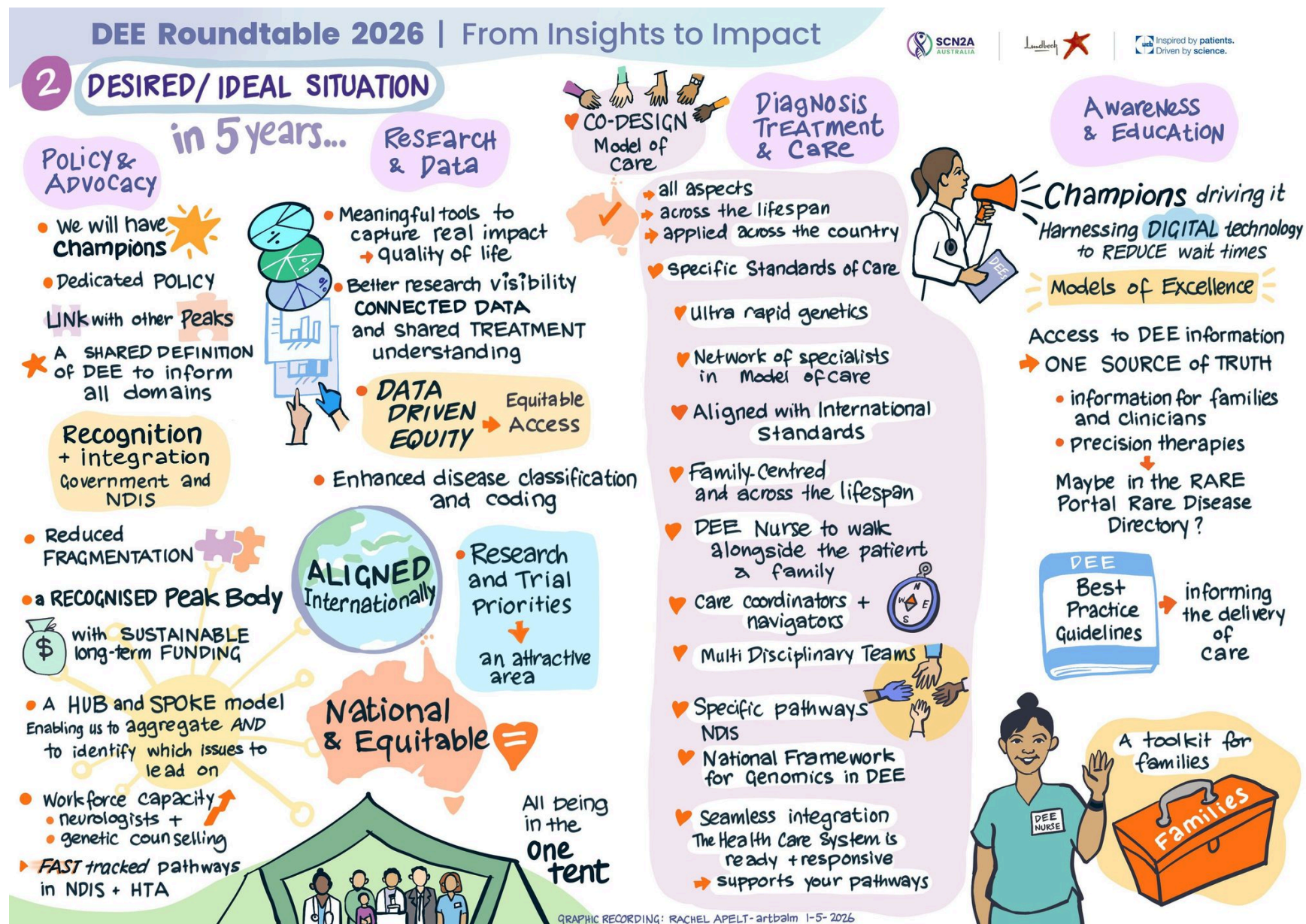
Appendix 4 Graphic recording: the shared vision

The closing illustration, drawing together the day's strongest theme: an umbrella organisation to represent all stakeholders in the DEE landscape.

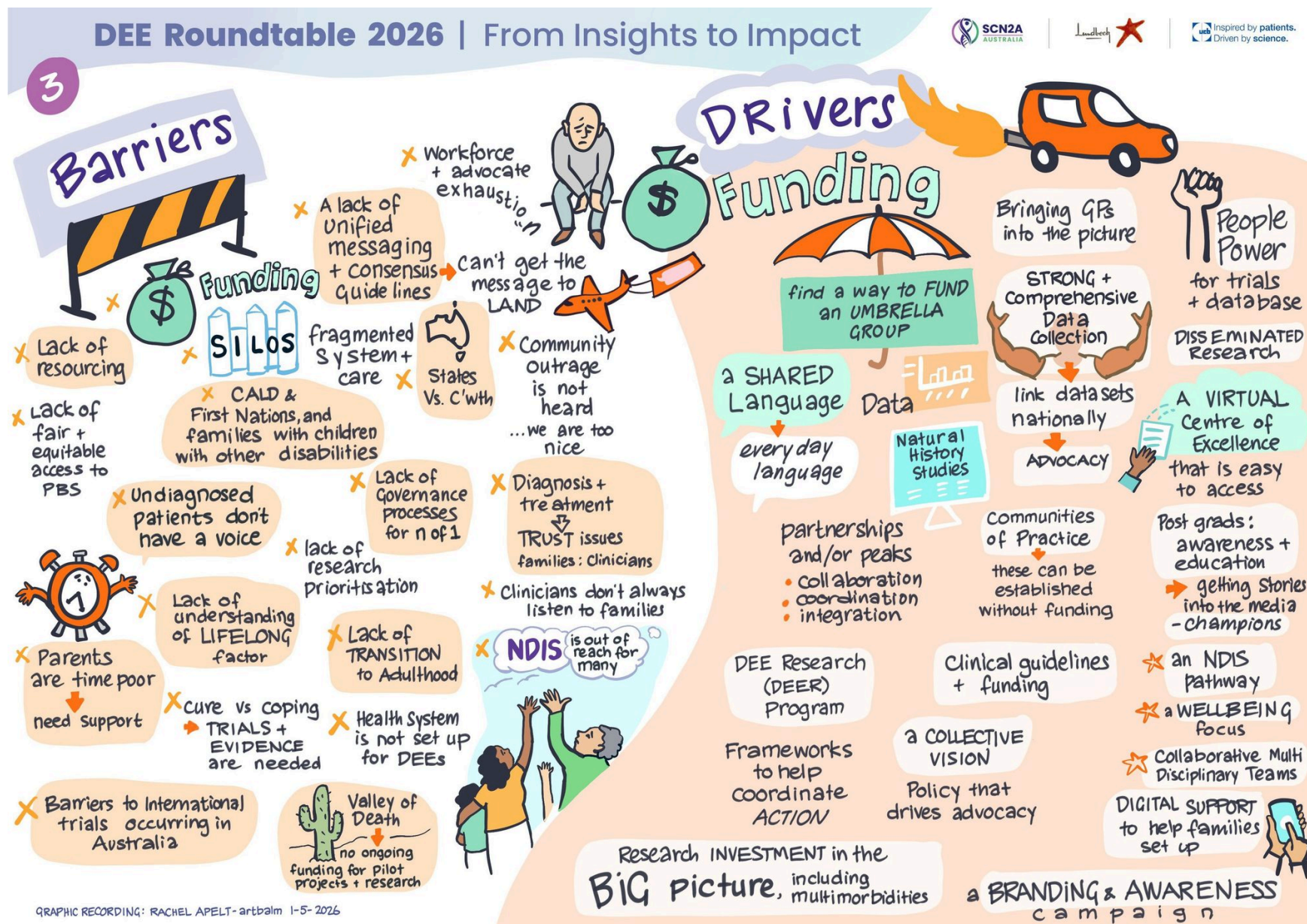
The list of organisations represented at the Roundtable is published in the Final Report, Appendix 2.



Appendix 1. Graphic recording, Workshop 1: the current situation. Rachel Apelt, artbalm, 1 May 2026.



Appendix 2. Graphic recording, Workshop 2: the desired and ideal situation in five years. Rachel Apelt, artbalm, 1 May 2026.



Appendix 3. Graphic recording, Workshop 3: barriers and drivers. Rachel Apelt, artbalm, 1 May 2026.



Appendix 4. Graphic recording: the shared vision. Establish an umbrella organisation to represent all stakeholders in the DEE landscape. Rachel Apelt, artbalm, 1 May 2026.